

Federal State Budgetary Institution
"Dmitry Rogachev National Medical
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Ministry of Health of Russia
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MINISTRY OF HEALTH OF THE RUSSIAN FEDERATION
Federal State Budgetary Institution
**'Dmitry Rogachev National Medical Research Centre
for Paediatric Haematology, Oncology and Immunology'**

Ministry of Health of the Russian Federation
(Federal State Budgetary Institution 'Dmitry Rogachev National Medical Research Centre for Paediatric Haematology, Oncology and
Immunology' of the Ministry of Health of the Russian Federation)

29 April 2026

№ 11

Moscow

MINUTES (CONCLUSION)
of the meeting of the Medical Sub-
Committee
held on 29 April 2026
(Order No. 247/19 of 27 December 2019)

The Medical Sub-Committee of the Dmitry Rogachev National Medical Research Centre for Paediatrics and Child Health (NMITS DGIO) of the
Ministry of Health of Russia, comprising (full name, position):

Chair of the sub-committee – D.V. Litvinov – Chief Medical Officer

Members of the subcommittee:

1. A.V. Pshonkin – Head of Department – Paediatric Oncologist (Short-Stay Ward)
2. A.E. Sysoev – paediatric oncologist (Neuro-Oncology Department)
3. Yu.A. Rodina – Head of Department – allergist and immunologist (Department of Immunology)
4. L.N. Shelikhova – Head of Department – haematologist (Haematopoietic Stem Cell Transplantation Department No. 1)

Agenda for the meeting of the Medical Sub-Committee: on recognising the patient as incurable, and on the need to provide palliative care.

Patient's full name: **BARAKHTENKO EGOR ANDREEVICH**

Date of birth: **24 April 2008** Age: **18 years** Gender: **Male**

Permanent address: Russian Federation, Sakha (Yakutia) Republic, Yakutsk, Sh Sergekhlahskoye, 5 km, house 45, flat 5A

Russian Federation passport, series: 98 22, number 005881, date of issue 16 June 2022, issued by 140-002 Ministry of Internal Affairs of the Republic
of Sakha (Yakutia)

Health insurance policy No. 1495199725000160, issued on 10 March 2015

Diagnoses:

Primary: Myelodysplastic syndrome with WT1 overexpression, mutation in the NRAS gene dated 21 November 24; first allogeneic haematopoietic stem cell transplant (HSCT) from an unrelated HLA-identical donor dated 1 April 25; first clinical-haematological and molecular remission from 29 April 2025. Secondary myeloid leukaemia following myelodysplastic syndrome from 3 July 25, internal tandem duplication in the FLT3 gene (FLT3/ITD), WT1 gene overexpression, BAALC; repeat allogeneic HSCT from a haploidentical donor (father) on 16 September 25; clinical-haematological and molecular remission from 14 October 25; molecular relapse (FLT3/ITD, overexpression of the WT1 gene, BAALC-39) from 13 November 25, persistence of minimal residual disease (MRD) (IFT) from 4 December 25; infusion of donor lymphocytes No. 1 (1.0×10^5) on 8 December 25; bone marrow relapse from 15 January 26. Administration of anti-CD33 CAR-NK lymphocytes on 8 April 2026 from a haploidentical donor (father). Progression from 23 April 2026. (C92.0 Acute myeloid leukaemia [AML])

Concomitant condition: Left aorto-mesenteric conflict (compression of the left renal vein). (N28.8 Other specified diseases of the kidney and ureter)

Complication of the primary condition:

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Complication of the primary condition: Induced haematopoietic aplasia (D70 Agranulocytosis)
condition: HSCT recipient (D84.8 Other specified immunodeficiency disorders)
T-cell stem cell transplant recipient (Z94.8 Presence of other transplanted organs and tissues)

Based on the findings of the multidisciplinary consultation held on 29 April 2026 for a patient with acute myeloid leukaemia on day +14 following CAR-T cell therapy, according to the follow-up examination (FUE dated 23 April 2026) the myelogram showed that the proportion of blast cells with morphological features of the myeloid lineage was 71.0% and 60.0% by points, respectively, MRD by flow cytometry: tumour population 51.2%, CD33+ expression in 100% of blast cells; persistence of anti-CD33 CAR-NK lymphocytes: 35.2% of NK cells in bone marrow, 40.4% of NK cells in peripheral blood. Taking into account the extent of treatment administered (chemotherapy, two allogeneic HSCT and CAR-T cell therapy), the patient's poor physical condition and refractoriness to the comprehensive therapy being administered, all curative options have now been exhausted. Due to the lack of alternative treatment methods, the child's condition has been deemed incurable. Further treatment can only be of a purely symptomatic nature, aimed at sustaining life and alleviating the patient's suffering. A discussion regarding the extremely poor prognosis has been held with the legal representative (the patient's father).

It has been decided to provide the following palliative care:

1. Suppressive therapy: prednisolone 10 mg twice daily, orally. In the event of increasing hepatosplenomegaly, leucocytosis in the peripheral blood, the possibility of administering courses of cytarabine 200 mg/m², vepezide 200 mg/m² or cyclophosphamide 150 mg/m² should be considered, alongside allopurinol 300 mg daily and infusion therapy.
2. Symptomatic treatment aimed at alleviating distressing symptoms, including pain relief on demand, with the use of opioid analgesics where necessary

Conclusions of the subcommittee:

1. In view of the test results obtained, the child's condition is progressing. The patient is deemed incurable.
2. It is recommended that the child receive palliative care at a healthcare facility in their place of residence.

Chair of the sub-committee

Signature

D.V. Litvinov

Members of the sub-committee

signature

A. V. Pshonkin

signature

A. E. Sysoev

signature

Yu. A. Homeland

signature

L. N. Shelikhova